

METROPOLITAN UNIVERSITY MEDICAL CENTER

INTEGRATED ELECTRONIC HEALTH RECORD - EMERGENCY DEPARTMENT ADMISSION CHART

STRICTLY CONFIDENTIAL - COMPLETED PATIENT PACKET

SECTION 1: EMERGENCY DEPARTMENT TRIAGE & DEMOGRAPHICS

PATIENT IDENTIFICATION REGISTRY

Full Legal Name: VALENTINE, CHLOE AMARA
Medical Record Number: MRN-77319-K
Account Number: FIN-22041-A
Date of Birth: 09/18/1996
Age / Gender: 29 Years / Female
Marital Status: Single
Home Address: 742 Appletree Lane, Apt 3C, Metropolis, NY 10011

ADMINISTRATIVE & INSURANCE REGISTRY

Primary Insurance: Vanguard PPO Premium Network
Policy ID Number: VNG-441290-A
Group Number: GRP-77142
Subscriber Name: Chloe A. Valentine
Emergency Contact: Marcus Valentine (Father)
Contact Phone: (555) 017-8834
Relationship to Pt: Next of Kin

TRIAGE INTAKE LOG

Triage Date: 05-23-2026
Arrival Timestamp: 02:14 AM
Mode of Arrival: Ambulatory (Private Vehicle via Self-Presentation)
Chief Complaint: "Severe, unbearable lower abdominal pain and stomach sickness since yesterday evening."
Triage Acuity Level: ESI Level 2 (Emergency Severity Index - High Risk)
Assigned Care Area: ED Acute Care Module - Bed 14
Triage Nurse: J. Vance, RN

SECTION 2: INITIAL TRIAGE VITALS & CLINICAL MEASUREMENTS

Timestamp: 02:18 AM
Blood Pressure: 138/84 mmHg (Mildly Elevated secondary to acute pain)
Heart Rate: 104 bpm (Sinus Tachycardia)
Respiratory Rate: 20 breaths/min
Temperature (Oral): 100.2 degrees F (37.9 degrees C) (Mild Fever)
Oxygen Saturation: 99% on Room Air
Pain Scale Score: 8/10 (Severe, localized to lower abdomen)
Height / Weight: 65 in (165 cm) / 134 lbs (60.8 kg)
Last Oral Intake (NPO): NPO since 08:00 PM yesterday evening (Liquid clear sips only)

SECTION 3: NURSE INTAKE & TRIAGE NARRATIVE NOTE

Log Timestamp: 02:25 AM
Authoring Clinician: J. Vance, RN

Patient is a 29-year-old female who presents to the Emergency Department complaining of severe, sharp, unrelenting lower abdominal pain. Patient states the pain began approximately 12 hours ago as a dull, generalized ache around the umbilicus, which subsequently migrated and localized sharply into the lower abdomen. She reports associated symptoms of persistent nausea, non-bilious emesis x2 at home, and a general lack of appetite.

On physical inspection, patient appears in acute distress, pale, and is guarding her abdomen. She is cooperative but visibly crying due to the severity of the pain. She denies any abnormal vaginal bleeding, dysuria, or recent changes in her bowel habits. Last menstrual period (LMP) began 12 days ago, described as regular and light.

***Ambiguous Symptom Annotation:** * Patient notes that her lower abdominal pain is uniquely exacerbated by taking deep breaths or shifting positions in the bed, but also mentions an unusual, vague "stabbing, fluttering sensation" deep in her pelvic cavity that radiates sporadically toward her right shoulder whenever she stands upright.

Patient has a current, patent 20-gauge peripheral intravenous line placed in her left antecubital fossa. Baseline blood draws obtained and dispatched to the central pathology lab. Fluid resuscitation initiated with Normal Saline at 125 mL/hr per standing ED protocol. Safety precautions established: bed in lowest position, side rails x2 raised, call bell within reach. Patient instructed to remain strictly NPO pending physician evaluation.

SECTION 4: EMERGENCY PHYSICIANS INITIAL ASSESSMENT & DIFFERENTIAL

Log Timestamp: 02:40 AM

Authoring Clinician: Dr. Christopher Cole, MD (ED Attending)

CHIEF COMPLAINT & HISTORY

The patient is a 29-year-old female with no significant past medical or surgical history who presents with acute lower abdominal pain, nausea, and mild fever. The timeline of symptoms is classic for a progressive inflammatory process. She states the pain is currently localized to the right lower quadrant and is an 8/10 in severity. She has been unable to tolerate solid food or liquids due to profound nausea.

PAST MEDICAL & SURGICAL HISTORY

Chronic Medical Illnesses: None documented.

Past Surgical History: None. No prior abdominal surgeries.

Social History: Denies tobacco use. Occasional social alcohol. Denies illicit drug use.

Family History: Non-contributory. No family history of inflammatory bowel disease or clotting disorders.

SYSTEMIC REVIEW (REVIEW OF SYSTEMS)

Constitutional: Positive for mild fever, chills, and fatigue.

Gastrointestinal: Positive for severe right lower quadrant abdominal pain, anorexia, nausea, and emesis. Negative for diarrhea, constipation, hematochezia, or melena.

Genitourinary: Negative for dysuria, urinary frequency, urgency, or abnormal vaginal discharge.

Neurological: Negative for headache, focal weakness, or syncope.

PHYSICAL EXAMINATION

General: Alert, oriented x4, appearing in acute discomfort, actively guarding the lower abdomen.

Vitals: Reviewed and verified. Tachycardic at 102 bpm. Oral temperature 100.2 degrees F.

Skin: Warm, dry, intact. No rashes or petechiae.

Cardio: Tachycardic, regular rhythm, normal S1 and S2. No murmurs, rubs, or gallops.

Lungs: Clear to auscultation bilaterally. Normal respiratory effort.

Abdomen: Marked tenderness to superficial and deep palpation within the right lower quadrant (RLQ). Positive rebound tenderness and positive involuntary guarding are present over McBurney's point. Positive Rovsing's sign (left lower quadrant palpation elicits severe pain in the right lower quadrant). Right-sided Psoas sign is highly positive. No palpable masses or organomegaly. Bowel sounds are hypoactive in all four quadrants.

Pelvic: Speculum exam deferred. Bimanual examination reveals localized right adnexal tenderness without exquisite cervical motion tenderness.

CLINICAL IMPRESSION & DIFFERENTIAL DIAGNOSIS

The clinical presentation is highly suggestive of an acute surgical abdomen. The migration of pain from the periumbilical region to the right lower quadrant combined with fever, peritoneal signs (rebound and guarding), and leukocytosis strongly suggests acute appendicitis. However, given the patient's age and anatomical structures, gynecological etiologies must be concurrently evaluated.

Primary Differential Diagnostics:

1. Acute Appendicitis (High index of suspicion based on peritoneal signs)
2. Right Ovarian Cyst Rupture or Hemorrhagic Ovarian Cyst Evaluation
3. Ovarian Torsion (Must rule out via pelvic imaging due to adnexal tenderness)

4. Pelvic Inflammatory Disease (PID) / Acute Adnexitis

CLINICAL PLAN

- Maintain strict NPO status.
- Administer IV fluid bolus, targeted analgesia, and antiemetics immediately.
- Order STAT Complete Blood Count, Comprehensive Metabolic Panel, and Beta-hCG to rule out ectopic pregnancy.
- Order STAT Computed Tomography (CT) of the abdomen and pelvis with intravenous contrast to differentiate between appendicitis and gynecological pathology.

SECTION 5: EMERGENCY DEPARTMENT MEDICATION ADMINISTRATION RECORD (MAR)

Ordered By: Dr. Christopher Cole, MD | Verified By: ED Pharmacy Registry

Timestamp	Medication Name	Dosage	Route	Administered By	Indication / Status
02:30 AM	Normal Saline 0.9%	1000 mL	IV	M. Alvarez, RN	Fluid Volume / Infusing
02:35 AM	Ondansetron HCl	4 mg	IV	M. Alvarez, RN	Acute Nausea / Completed
02:45 AM	Morphine Sulfate	4 mg	IV	M. Alvarez, RN	Severe Pain / Completed
03:50 AM	Acetaminophen IV	1000 mg	IV	M. Alvarez, RN	Pyrexia / Completed
04:15 AM	Piperacillin-Tazo	3.375 g	IV	M. Alvarez, RN	Prophylactic / Infusing

CLINICIAN TIMESTAMP INCONSISTENCY & CONFLICT LOG:

Nursing Administration Log entry at 03:15 AM by M. Alvarez, RN notes: "Patient is resting quietly, completely pain-free, sleeping comfortably on her side. Abdomen is soft, non-tender to light palpation throughout all quadrants; patient states pain is now 0/10."

This documentation directly conflicts with Dr. Cole's re-evaluation note logged at 03:30 AM, which states: "Patient continues to experience severe, intractable right lower quadrant pain despite 4 mg of IV Morphine. Re-examination demonstrates persistent involuntary guarding and exquisite tenderness over McBurney's point. Awaiting emergent CT scan imaging results."

SECTION 6: LABORATORY DIAGNOSTIC PROFILES

Specimen Collection Date: May 23, 2026 | Collection Time: 02:22 AM

Lab Facility: MUMC Emergency Core Pathology Laboratory

COMPLETE BLOOD COUNT (CBC) WITH DIFFERENTIAL

Component Test	Result Value	Reference Range	Status Flags
White Blood Cell (WBC)	16.4 x10 ³ / µL	4.5 - 11.0 x10 ³ / µL	HIGH (Abnormal)
Red Blood Cell (RBC)	4.35 x10 ⁶ / µL	3.80 - 5.10 x10 ⁶ / µL	Normal
Hemoglobin (Hgb)	13.2 g/dL	11.7 - 15.5 g/dL	Normal
Hematocrit (Hct)	39.5 %	35.0 - 45.0 %	Normal
Platelet Count	285 x10 ³ / µL	150 - 400 x10 ³ / µL	Normal
Neutrophils, Absolute	13.8 x10 ³ / µL	1.8 - 7.7 x10 ³ / µL	HIGH (Abnormal)
Lymphocytes, Absolute	1.4 x10 ³ / µL	1.0 - 4.8 x10 ³ / µL	Normal
Bands (Immature Neutro)	8 %	0 - 5 %	HIGH (Left Shift)

COMPREHENSIVE METABOLIC PANEL (CMP) & BIOMARKERS

Component Test	Result Value	Reference Range	Status Flags
Serum Sodium	139 mEq/L	136 - 145 mEq/L	Normal
Serum Potassium	3.8 mEq/L	3.5 - 5.1 mEq/L	Normal
Serum Chloride	102 mEq/L	98 - 107 mEq/L	Normal

Component Test	Result Value	Reference Range	Status Flags
Carbon Dioxide (Bicarbonate)	24 mEq/L	22 - 29 mEq/L	Normal
Blood Urea Nitrogen (BUN)	14 mg/dL	7 - 20 mg/dL	Normal
Serum Creatinine	0.72 mg/dL	0.50 - 1.10 mg/dL	Normal
Serum Glucose (Random)	108 mg/dL	70 - 140 mg/dL	Normal
Total Bilirubin	0.6 mg/dL	0.2 - 1.2 mg/dL	Normal
Alkaline Phosphatase (ALP)	64 U/L	30 - 120 U/L	Normal
Aspartate Aminotransferase	22 U/L	10 - 40 U/L	Normal
Alanine Aminotransferase	19 U/L	7 - 56 U/L	Normal
C-Reactive Protein (CRP)	4.2 mg/dL	< 0.8 mg/dL	HIGH (Inflam)
Qualitative Serum Beta-hCG	Negative	Negative	Normal (Non-Preg)

SECTION 7: IMAGING & RADIOLOGY REPORT

DOCUMENT TYPE: Computed Tomography (CT) of the Abdomen and Pelvis with IV Contrast

EXAMINATION DATE: May 23, 2026

TIMESTAMP OF EXAM: 03:45 AM

ACCESSION NUMBER: RAD-CT-99412-V

ORDERING PHYSICIAN: Dr. Christopher Cole, MD

INTERPRETATION BY: Dr. Sarah Vance, MD (Department of Diagnostic Radiology)

CLINICAL INDICATION

A 29-year-old female presenting with acute right lower quadrant abdominal pain, fever, and leukocytosis. Evaluate for acute appendicitis vs. acute gynecological pathology.

TECHNIQUE

Helical CT scanning of the abdomen and pelvis was performed from the level of the dome of the diaphragm through the pubic symphysis following the uncomplicated administration of 100 mL of Omnipaque intravenous contrast material. Axial, sagittal, and coronal reformatted images were reviewed.

FINDINGS

Gastrointestinal Tract:

The appendix is visualized in the right lower quadrant and is distinctly abnormal. It demonstrates clear dilatation, measuring up to 9.5 mm in outer diameter (normal is less than 6 mm). There is prominent circumferential thickening and hyperenhancement of the appendiceal wall, creating a classic "target sign" appearance. Marked surrounding mesenteric fat stranding and a small amount of localized free fluid are noted within the right lower quadrant, localized entirely around the cecal tip. No distinct appendicolith is seen within the lumen. The cecal wall displays reactive thickening. The loops of the small bowel and the remainder of the large colon are normal in caliber, showing normal wall enhancement without evidence of mechanical bowel obstruction or pneumoperitoneum.

Genitourinary Tract / Pelvis:

The uterus is normal in size, shape, and position, displaying homogeneous endometrial enhancement. The left ovary is normal in appearance, measuring 3.2 cm, with normal stromal blood flow. The right ovary is well-visualized, measuring 3.4 cm, and contains a small, thin-walled, simple unilocular fluid collection measuring 1.8 cm, which is completely consistent with a benign, non-ruptured physiologic follicular cyst. There is no evidence of abnormal right adnexal mass enlargement, complex pelvic fluid, or signs of ovarian torsion.

Solid Organs:

The liver, spleen, gallbladder, pancreas, and adrenal glands are normal in size, morphology, and enhancement characteristics. No focal masses, cysts, or lacerations are detected. Both kidneys are symmetric in size with appropriate parenchymal nephrograms; no hydronephrosis or radiopaque calculi are visualized bilaterally.

Lymphatics / Vasculature:

There is mild, reactive mesenteric lymphadenopathy clustered within the right lower quadrant. The abdominal aorta and inferior vena cava are normal in caliber.

DIAGNOSTIC IMPRESSION

1. CT FINDINGS STRONGLY INDEPENDENTLY CONFIRM ACUTE APPENDICITIS: Dilated, thick-walled appendix measuring 9.5 mm with prominent localized hyperenhancement, regional mesenteric fat stranding, and a small layer of localized free fluid. No signs of appendiceal perforation or abscess formation.
2. SMALL, BENIGN PHYSIOLOGIC RIGHT OVARIAN FOLLICULAR CYST (1.8 CM): Completely intact, thin-walled, with no features of rupture, hemorrhage, or associated adnexal torsion.

SECTION 8: FINAL DIAGNOSIS SUMMARY & SURGICAL CARE COORDINATION

Log Timestamp: 04:45 AM

Authoring Clinician: Dr. Christopher Cole, MD

Following a comprehensive clinical evaluation, lab review, and multi-planar diagnostic imaging, the patient's presentation has reached definitive diagnostic resolution.

PRIMARY EMERGENCY DIAGNOSIS:

Acute Appendicitis without Perforation (ICD-10 Code: K35.80)

SECONDARY INCIDENTAL DIAGNOSIS:

Benign Localized Right Ovarian Cyst, Unspecified (ICD-10 Code: N83.201)

CASE DISPOSITION & COORDINATION

The findings of acute appendicitis were discussed immediately with the patient. Surgical consultation was requested urgently from the General Surgery On-Call Team (Dr. Richard Marcus, MD). Dr. Marcus reviewed the clinical chart, evaluated the patient at the bedside, confirmed the diagnosis, and recommended immediate inpatient admission to the surgical service for an emergent laparoscopic appendectomy.

The patient has consented to the operative procedure. She remains stable, on continuous IV fluid support, and has been administered broad-spectrum prophylactic antibiotics (Piperacillin-Tazobactam). She is currently being prepared for safe transport to the main Operating Room (OR Block 3).

SECTION 9: PATIENT DISCHARGE & INPATIENT TRANSFER INSTRUCTIONS

Note: These instructions were reviewed with the patient and provided to the floor nursing team for the patient's post-operative recovery transition.

DIETARY RESTRICTIONS

You are to remain strictly NPO (nothing by mouth). Do not drink any water, ice chips, or eat any food until after your surgical operation is completed and your surgical team explicitly tells you it is safe to resume eating.

ACTIVITY RESTRICTIONS

You must remain on bed rest until your surgery. After surgery, you will be encouraged to walk gently around the hospital floor under the supervision of your nurse to assist with recovery and prevent blood clots. Avoid any strenuous activity, heavy lifting over 10 pounds, or intense exercise until cleared by your surgeon at your follow-up clinic visit.

MEDICATION DIRECTIONS

Your emergency department care team has administered medications intravenously to control your pain and nausea, alongside antibiotics to treat the inflammation in your appendix. After your appendectomy, your surgical team will provide you with a new, specific list of oral discharge prescriptions for pain management.

WARNING SIGNS TO REPORT IMMEDIATELY

If you experience any of the following symptoms during your hospital stay or immediately after your upcoming discharge, alert your healthcare providers or seek emergency care immediately:

- A sudden, severe worsening of abdominal pain or a hard, rigid abdomen.
- A new or worsening fever over 101 degrees F.
- Persistent, uncontrollable nausea or vomiting.
- Severe dizziness, lightheadedness, or feeling like you might pass out.
- Redness, severe swelling, warmth, or foul-smelling drainage from your surgical incision sites.

SECTION 10: POST-OPERATIVE OUTPATIENT PRESCRIPTION REGIMEN

The following pending prescriptions have been generated by the surgical service and queued to the hospital discharge pharmacy system, to be filled and finalized upon the completion of the patient's laparoscopic operation.

Prescription 1:

Medication Name: Oxycodone-Acetaminophen (Percocet)

Formulation / Strength: 5 mg / 325 mg Oral Tablet

Dosing Frequency: Take 1 tablet every 6 hours by mouth as needed for severe post-operative pain.

Quantity Authorised: 12 Tablets

Refills Allowed: Zero (0)

Prescribing Physician: Dr. Richard Marcus, MD (General Surgery)

Prescription 2:

Medication Name: Ibuprofen

Formulation / Strength: 600 mg Oral Tablet

Dosing Frequency: Take 1 tablet every 8 hours by mouth with food for mild-to-moderate inflammatory pain control.

Quantity Authorised: 20 Tablets

Refills Allowed: Zero (0)

Prescribing Physician: Dr. Richard Marcus, MD (General Surgery)

Prescription 3:

Medication Name: Docusate Sodium (Colace)

Formulation / Strength: 100 mg Oral Capsule

Dosing Frequency: Take 1 capsule twice daily by mouth until normal bowel function returns to prevent straining.

Quantity Authorised: 30 Capsules

Refills Allowed: One (1)

Prescribing Physician: Dr. Richard Marcus, MD (General Surgery)

MANDATED FOLLOW-UP PROTOCOL

You are scheduled to attend a post-operative evaluation appointment at the General Surgery Outpatient Clinic with Dr. Richard Marcus within 10 to 14 days following your surgery. Please call the clinic coordinator at (555) 019-4421 to confirm the date and time.

***** END OF EMERGENCY CHART PACKET - ELECTRONICALLY SIGNED AND VERIFIED *****